

Dynapharm

DYCOSIC TABLET

MAL18056180AZ

CONTENT

Orphenadrine citrate 35 mg
Paracetamol 450 mg.

PHARMACODYNAMICS

Orphenadrine, which is a congener of diphenhydramine without sharing its soporific effect, is a tertiary amine antimuscarinic with actions and uses similar to those of trihexyphenidyl. It also has weak antihistaminic and local anaesthetic properties.

Paracetamol is a centrally acting analgesic and antipyretic with minimal anti-inflammatory properties.

PHARMACOKINETICS

Orphenadrine is readily absorbed from the gastro-intestinal tract, and very readily absorbed following intramuscular injection. It is rapidly distributed in tissues and most of a dose is metabolised and excreted in the urine along with a small proportion of unchanged drug. A half-life of 14 hours has been reported.

Paracetamol is rapidly absorbed following oral administration.

Paracetamol absorption takes place mainly in the small intestine and therefore the rate of absorption depends on the rate of gastric emptying.

The presence of food in the stomach has also been reported to reduce the rate of absorption of paracetamol. Alterations in gastric pH have no appreciable effect on paracetamol absorption.

Peak plasma concentrations are reached 1 hour after absorption. The plasma half-life is 1 to 3 hours.

Paracetamol penetrates the brain and is present in breast milk of human.

Paracetamol is metabolized by the microsomal enzyme system of the liver. This metabolism is mainly to the glucuronide and sulphate conjugates, accounting for approximately 49% and 26% of the ingested dose respectively. About 4% is excreted as free paracetamol. Other minor pathways include the production of catechol derivatives and cysteine conjugates (via glutathione). Paracetamol excretion is rapid and occurs via the urine.

INDICATIONS

Tension headache, occipital headaches associated with spasm of skeletal muscles in the region of the head and neck. Acute and traumatic conditions of the limbs and trunk: sprains, strains, whiplash injuries, acute torticollis, prolapsed intervertebral disc.

DOSAGE

Oral administration.
2 tablets three times daily.

CONTRAINDICATIONS

Glaucoma, prostatic hypertrophy or obstruction at the bladder neck, myasthenia gravis, oesophageal spasm and pyloric or duodenal obstruction.
Hypersensitivity to paracetamol or orphenadrine citrate.

PRECAUTIONS

Orphenadrine citrate should be used with caution in patients with tachycardia, cardiac decompensation, coronary insufficiency or cardiac arrhythmias.

Paracetamol should be used with caution in patients with hepatic or renal dysfunction.

Concomitant treatment with other medicines that contain orphenadrine or paracetamol is not recommended.

Safety of continuous long-term therapy with orphenadrine has not been established. Therefore if orphenadrine is prescribed for prolonged use, periodic monitoring of blood, urine and liver function is recommended.

Paediatric use

DYCOSIC is not recommended for children under 12 years of age.

Use in the elderly

The elderly should be advised to take a reduced dosage as they may be more susceptible to anti-cholinergic side effects at regular doses.

WARNING

- Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

This preparation contains PARACETAMOL.
Do not take any other paracetamol containing medicines at the same time.

Effect on ability to drive or operate machinery

Orphenadrine may impair the ability of the patient to engage in potentially hazardous activities such as operating machinery or driving a motor vehicle; ambulatory patients should therefore be cautioned accordingly.

INTERACTIONS

Interactions have been reported between orphenadrine and phenothiazines and other drugs with anti-muscarinic properties. Concomitant use with alcohol or other CNS depressants should be avoided.

Anticoagulant dosage may require reduction if paracetamol medication is prolonged.

Paracetamol absorption is increased by medicines that increase gastric emptying, e.g. metoclopramide, and decreased by medicines that decrease gastric emptying, e.g. propantheline, antidepressants with anticholinergic properties and narcotic analgesics. Paracetamol may increase chloramphenicol concentrations. The likelihood of paracetamol toxicity may be increased by the concomitant use of enzyme inducing agents such as alcohol or anticonvulsant medicines.

PREGNANCY AND LACTATION

Use in pregnancy

DYCOSIC is not recommended for use during pregnancy.

Use in lactation

DYCOSIC should not be taken during lactation as orphenadrine and paracetamol are excreted into breast milk.

SIDE EFFECTS

Adverse effects are mainly due to the anti-cholinergic action of orphenadrine and are usually associated with higher doses.

Orphenadrine citrate**More common reactions**

The known adverse effects include; dryness of the mouth, tachycardia, palpitation, urinary hesitancy or retention, blurred vision, dilation of the pupils, increased ocular tension, weakness, nausea, headache, dizziness, constipation and drowsiness. These effects can usually be eliminated by reducing the dose

Less common reactions

Sedation, skin rashes and other allergic reactions are very uncommon adverse effects.

Infrequently an elderly patient may experience some degree of mental confusion. Very rare cases of aplastic anaemia associated with the use of orphenadrine have been reported.

Paracetamol

Reports of adverse reactions are rare. Although the following reactions have been reported, a causal relationship to the administration of paracetamol has been neither confirmed nor refuted; dyspepsia, nausea, allergic and haematological reactions.

Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

OVERDOSE

No specific information is available on overdosage with DYCOSIC.

Overdose of paracetamol can result in severe liver damage and sometimes acute renal tubular necrosis.

Symptoms and Signs

Orphenadrine overdose: Known symptoms of overdose with orphenadrine include tachycardia, excitement, confusion and delirium leading to coma. Convulsions, dilated pupils and urinary retention may occur.

Paracetamol overdose: Toxic symptoms following an overdose with paracetamol include vomiting, abdominal pain, hypotension, sweating, central stimulation with exhilaration and convulsions in children, drowsiness, respiratory depression, cyanosis and coma.

In adults, hepatotoxicity may occur after ingestion of a single dose of paracetamol 10 to 15g; a dose of 25g or more is potentially fatal.

Symptoms during the first two days of acute poisoning by paracetamol do not reflect the potential seriousness of the intoxication. Major manifestations of liver failure such as jaundice, hypoglycaemia and metabolic acidosis may take at least three days to develop.

Treatment

Prompt treatment is essential even when there are no obvious symptoms.

In cases of overdosage, methods of reducing absorption of ingested medicine are important.

Prompt administration of activated charcoal 50 g in 150 mL of water and 150 mL sorbitol 50% solution by mouth may reduce absorption. It is recommended that intravenous fluids such as normal saline be given concurrently. Gastric lavage is indicated if the patient is unwilling or unable to drink an activated charcoal/sorbitol mixture.

If the history suggests that paracetamol 150mg/kg body weight or 15 g total or more has been ingested, administer the following antidote:

Intravenous acetylcysteine 20%: Administer acetylcysteine immediately without waiting for positive urine test or plasma level results if 8 hours or less since overdose ingestion. Initial dose 150 mg/kg over 15 minutes, followed by continuous infusion of 50 mg/kg in glucose 5% 500 mL over four hours and 100 mg/kg in glucose 5% 1 L over 16 hours. If more than eight hours have elapsed since the overdose was taken, the antidote may be less effective.

Convulsions and delirium respond to relatively large doses of diazepam, preferably by mouth. Adequate hydration of the patient is important.

PRESENTATION

Tablets (white, oblong, convex and uncoated)

PACKING

Blister pack of 50 x 10 tablets per box.

STORAGE CONDITIONS

JAUHI UBAT DARIPADA KANAK-KANAK

KEEP OUT OF REACH OF CHILDREN

Store in dry place below 30°C

Protect From Light

MANUFACTURER & PRODUCT REGISTRATION HOLDER

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